



CLINICIAN SUMMARY

What this profile may indicate.

Self-report results show moderate-range depression indicators, moderate-range anxiety indicators, and mild-range anger indicators, with moderate-range global distress. The pattern is concentrated in dysphoria, unsustained effort, negative cognition, and fatigue rather than evenly distributed across domains. Critical-screen responses do not show a non-zero item in this record; routine clinical verification remains appropriate. These findings are hypotheses for a licensed clinician to test against interview data, history, context, and observed functioning; they do not establish a diagnosis.

RECORD	FF-TEST-2388-B
COMPLETED	Yesterday · 4:06 PM
COVERAGE	105 of 105
SOURCE	e-QPASS synthetic fixture

01 Signal profile

Scored e-QPASS output stays authoritative.

DEPRESSION 51 MODERATE	ANXIETY 42 MODERATE	ANGER 29 MILD	GLOBAL INDEX 141 MODERATE
SAFETY ROUTING No non-zero critical-screen response in this record			0 HIGHLIGHTED SOURCE RESPONSES

02 Evidence-linked hypotheses

Interview hypotheses, not diagnostic conclusions.

01 Broad negative-affect burden HIGH CONFIDENCE Moderate depression and moderate anxiety indicators co-occur with a moderate global index. This convergence may reflect a broader pattern, but duration, functional impact, and context still require direct assessment. Depression · 51 Anxiety · 42 GPI · 141	02 Motivation, cognition, and fatigue may converge MODERATE CONFIDENCE The depression-related profile is distributed across several constructs rather than isolated to one signal. Clarify medical, sleep, medication, substance, situational, and temporal contributors before drawing conclusions. Negative cognition · 14 Dysphoria · 9 Unsustained effort · 8	03 Worry-led anxiety pattern MODERATE CONFIDENCE Moderate anxiety indicators are distributed across apprehension, interpersonal concerns, and physiological arousal. Clarify persistence, triggers, avoidance, and functional impact. Anxiety · 42 Apprehension · 13
---	---	--

03 Questions for the next conversation

Use context and clinical judgment to test the pattern.

01 What changed around the onset of reduced interest, energy, motivation, or self-evaluative strain?	02 Which situations most reliably activate worry, avoidance, interpersonal tension, or physiological arousal?
03 How long has this broader pattern been present, and which aspects most disrupt daily functioning?	04 Which relationships, routines, coping strategies, and other protective factors are currently helping?

CLINICAL BOUNDARY

This structured decision-support summary is derived from self-report scores. It does not diagnose, prescribe, determine level of care, or replace interview data, history, functional assessment, direct safety assessment, or licensed clinical judgment.

Legal review pending · ff-clinical-disclaimer/draft-2026-08

APPROVED BY

J. Alvarez, PsyD
Not recorded

PERL-CLINICIAN-REPORT/1.0

deterministic-calibration · cal-0.9.3
Source a9ddb9a430f · Artifact 68350b970c7a